HOUSE OF REPRESENTATIVES - FLOOR VERSION

STATE OF OKLAHOMA

2nd Session of the 60th Legislature (2026)

COMMITTEE SUBSTITUTE
FOR
HOUSE BILL NO. 3644

By: Stinson and **Archer** of the House

and

Hines of the Senate

COMMITTEE SUBSTITUTE

[venous thromboembolism – hospitals – training – State Department of Health – registry – requirements – reports – definitions – rules – nursing care – adult daycare – information – nursing aides – noncodification – effective date – emergency]

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. NEW LAW A new section of law not to be codified in the Oklahoma Statutes reads as follows:

This act shall be known and may be cited as the "Blake Burgess Act".

SECTION 2. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 1-630 of Title 63, unless there is created a duplication in numbering, reads as follows:

Each hospital with an emergency department and each ambulatory surgical center shall:

1. Develop and implement policies and procedures for the rendering of appropriate medical attention for persons at risk of forming venous thromboembolisms which reflect evidence-based best practices relating to, at a minimum:

a. assessing patients for risk of venous thromboembolism using a nationally recognized risk assessment tool, and

b. treatment options for a patient diagnosed with venous thromboembolism; and

2. Train all nonphysician personnel at least annually on the policies and procedures developed under this section. For purposes of this section, the term "nonphysician personnel" means all personnel of the licensed facility working in clinical areas and providing patient care, except those persons licensed as health care practitioners.

SECTION 3. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 1-631 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. The State Department of Health shall contract with the state-designated health information exchange to establish and maintain a statewide venous thromboembolism registry to ensure that the performance measures required to be submitted under subsection B of this section are maintained and available for use to improve or modify the venous thromboembolism care system, ensure compliance with nationally recognized guidelines, and monitor venous thromboembolism patient outcomes.

B. Beginning July 1, 2027, each hospital with an emergency department shall regularly report to the statewide venous thromboembolism registry information containing nationally recognized venous thromboembolism measures and data on the incidence and prevalence of venous thromboembolisms. Such data shall include the following information:

1. The number of venous thromboembolisms identified and diagnosed;

2. The age of the patient;

3. The ZIP code of the patient;

4. The sex of the patient;

5. Whether the patient is a resident of a licensed nursing or assisted living facility;

6. Whether the venous thromboembolism was fatal;

7. How the diagnosis was made, such as by using imaging modalities; and

8. The treatment that was recommended for the venous thromboembolism.

C. The Department shall require the state-designated health information exchange to use a nationally recognized platform to collect data from each hospital with an emergency department on the performance measures required under subsection B of this section. The state-designated health information exchange shall provide to the Department regular reports on the data collected.

D. By June 1, 2027, the Department shall submit to the Governor, the President Pro Tempore of the Oklahoma State Senate, and the Speaker of the Oklahoma House of Representatives a detailed report on the incidence of venous thromboembolism using inpatient and outpatient data for services provided between July 1, 2026, and June 30, 2027. The report shall provide analyses of all of the following:

1. Age category, initial primary diagnosis and procedure, and secondary diagnoses, readmission rates for inpatients, admission rates for venous thromboembolism for which the patient had an ambulatory surgery procedure, and emergency department visits for venous thromboembolism linked to any previous admission;

2. Whether the venous thromboembolism was present upon admission;

3. The incidence of venous thromboembolism procedures reported on the agency's website; and

4. The principal payor, the sex of the patient, and the patient's discharge status.

E. The state-designated health information exchange operating the registry shall only use or publish information from the registry for the purposes of advancing medical research or medical education in the interest of reducing morbidity or mortality.

SECTION 4. AMENDATORY 63 O.S. 2021, Section 1-890.2, is amended to read as follows:

Section 1-890.2. As used in the Continuum of Care and Assisted Living Act:

1. "Assisted living center" means any home or establishment offering, coordinating or providing services to two or more persons who:

a. are domiciled therein,

b. are unrelated to the operator,

c. by choice or functional impairments, need assistance with personal care or nursing supervision,

d. may need intermittent or unscheduled nursing care,

e. may need medication assistance, and

f. may need assistance with transfer and/or ambulation;

2. "Board" means the State Board of Health;

3. "Commissioner" means the Commissioner of Health;

4. "Continuum of care facility" means a home, establishment or institution providing nursing facility services as defined in Section 1-1902 of this title and one or both of the following:

a. assisted living center services as defined in the Continuum of Care and Assisted Living Act, and

b. adult day care center services as defined in Section 1-872 of this title; ~~and~~

5. "Department" means the State Department of Health;

6. "Pulmonary embolism (PE)" means a condition in which part of the clot breaks off and travels to the lungs, possibly causing death; and

7. "Venous thromboembolism (VTE)" means deep vein thrombosis (DVT), which is a blood clot located in a deep vein, usually in the leg or arm. The term can be used to refer to DVT, pulmonary embolism, or both.

SECTION 5. AMENDATORY 63 O.S. 2021, Section 1-890.3, as amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025, Section 1-890.3), is amended to read as follows:

Section 1-890.3. A. The State Commissioner of Health shall promulgate rules necessary to implement the provisions of the Continuum of Care and Assisted Living Act. Such rules shall include, but shall not be limited to:

1. A uniform comprehensive resident screening instrument to measure the needs and capabilities of residents in all settings and to determine appropriate placements of residents;

2. Physical plant requirements meeting construction and life safety codes, with provisions accommodating resident privacy and independence in assisted living centers and in assisted living components of continuum of care facilities based on the variable capabilities of residents;

3. Staffing levels responsive to the variable needs of residents, with provisions for sharing of staff between components in a continuum of care facility;

4. Minimum standards for resident care including, but not limited to, standards pertaining to medical care and administration of medications. Standards pertaining to medication administration shall, at a minimum, require the assisted living center or continuum of care facility to:

a. provide or arrange qualified staff to administer medications based on the needs of residents,

b. follow medication administration orders from a qualified health care provider,

c. ensure that medications are reviewed monthly by a Registered Nurse or pharmacist and quarterly by a consultant pharmacist,

d. maintain medication administration records and document all medication administration in such records, and

e. have medication storage and disposal policies;

5. Standards for measuring quality outcomes for residents;

6. Provisions for individualized services chosen by and designed for each resident;

7. Provisions to prohibit facility staff from disclosing a resident's financial information to third parties without written consent of the resident or the designated representative of the resident;

8. Procedures for inspections and investigations of licensed entities to ensure compliance with the Continuum of Care and Assisted Living Act and rules promulgated by the Commissioner;

9. Enumeration of resident rights and responsibilities to be observed by each facility and its staff. Such resident rights shall include the freedom of choice regarding any personal attending physicians and all other providers of medical services and supplies, providing that the minimum standards are met by the provider pursuant to the Continuum of Care and Assisted Living Act, without a financial penalty or fee charged by the assisted living center;

10. Provisions for a surety bond or deposit from each applicant in an amount sufficient to guarantee that obligations to residents will be performed, with provisions for reduction or waiver of the

surety bond or deposit when the assets of the applicant or its contracts with other persons are sufficient to reasonably ensure the performance of its obligations;

11. Assisted living facilities shall provide a consumer information pamphlet to residents upon admission. The pamphlet shall contain information about venous thromboembolism (VTE), risk factors, and how residents can recognize the signs and symptoms of VTE;

12. Provisions for the development of a consumer guide or similar resource to be posted on the Internet website of the State Department of Health to assist individuals and families in understanding the services provided by assisted living centers and to compare and select a facility;

~~12.~~ 13. Provisions for posting results of routine inspections and any complaint investigations of each assisted living center on the Internet website of the Department. Such information shall be regularly updated to include the facility's plan of correction and to indicate when a violation of a licensing regulation was corrected by the facility; and

~~13.~~ 14. Provisions requiring execution of a plan of care and a resident service contract with the resident or resident's representative.

B. The nursing care service of a continuum of care facility shall be subject to the requirements, procedures and remedies set

out in the Nursing Home Care Act, including provisions relating to resident rights.

C. The adult day care component of a continuum of care facility shall be subject to requirements and procedures specified under the Adult Day Care Act.

SECTION 6. AMENDATORY 63 O.S. 2021, Section 1-1951, is amended to read as follows:

Section 1-1951. A. The State Department of Health shall have the power and duty to:

1. Issue certificates of training and competency for nurse aides;

2. Approve training and competency programs including, but not limited to, education-based programs and employer-based programs, including those programs established pursuant to Section 223.1 of Title 72 of the Oklahoma Statutes;

3. Determine curricula and standards for training and competency programs. The Department shall require such training to include a minimum of ten (10) hours of training in the care of Alzheimer's patients; and for direct care staff, recognizing signs and symptoms of venous thromboembolism (VTE) and techniques for providing an emergency response;

4. Establish and maintain a registry for certified nurse aides and for nurse aide trainees;

5. Establish categories and standards for nurse aide certification and registration, including feeding assistants as defined in 42 CFR Parts 483 and 488;

6. Exercise all incidental powers as necessary and proper to implement and enforce the provisions of this section; and

7. Suspend or revoke any certification issued to any nurse aide, if:

a. the nurse aide is found to meet any of the requirements contained in subsection D of Section 1-1947 of this title,

b. the nurse aide is found to meet any of the requirements contained in subsection C of Section 1-1950.1 of this title, or

c. the nurse aide is found to have committed abuse, neglect or exploitation of a resident or misappropriation of resident or client property pursuant to the requirements contained in paragraph 7 of subsection ~~D~~ E of this section. The action to revoke or suspend may be included with the filing of any action pursuant to the requirements of paragraph 7 of subsection D of this section.

B. The State Board of Health shall promulgate rules to implement the provisions of this section and shall have power to assess fees.

1. Each person certified as a nurse aide pursuant to the provisions of this section shall be required to pay certification and recertification fees in amounts to be determined by the State Board of Health, not to exceed Fifteen Dollars ($15.00).

2. In addition to the certification and recertification fees, the State Board of Health may impose fees for training or education programs conducted or approved by the Department, except for those programs operated by the Oklahoma Department of Veterans Affairs.

3. All revenues collected as a result of fees authorized in this section and imposed by the Board shall be deposited into the Public Health Special Fund.

C. Only a person who has qualified as a certified nurse aide and who holds a valid current nurse aide certificate for use in this state shall have the right and privilege of using the title Certified Nurse Aide and to use the abbreviation CNA after the name of such person. Any person who violates the provisions of this section shall be subject to a civil monetary penalty to be assessed by the Department.

D. A person qualified by the Department as a certified nurse aide shall be deemed to have met the requirements to work as a home health aide pursuant to the provisions of the Home Care Act and shall require no further licensure for performing services within the scope of practice of home health aides.

E. 1. The State Department of Health shall establish and maintain a certified nurse aide, nurse aide trainee and feeding assistant registry that:

a. is sufficiently accessible to promptly meet the needs of the public and employers, and

b. provides a process for notification and investigation of alleged abuse, exploitation or neglect of residents of a facility or home, clients of an agency or center, or of misappropriation of resident or client property.

2. The registry shall contain information as to whether a nurse aide has:

a. successfully completed a certified nurse aide training and competency examination,

b. met all the requirements for certification, or

c. received a waiver from the Board.

3. The registry shall include, but not be limited to, the following information on each certified nurse aide or nurse aide trainee:

a. the full name of the individual,

b. information necessary to identify each individual. Certified nurse aides and nurse aide trainees shall maintain with the registry current residential addresses and shall notify the registry, in writing, of any change of name. Notification of change of name

shall require certified copies of any marriage license or other court document which reflects the change of name. Notice of change of address or telephone number shall be made within ten (10) days of the effected change. Notice shall not be accepted over the phone,

c. the date the individual became eligible for placement in the registry, and

d. information on any finding of the Department of abuse, neglect or exploitation by the certified nurse aide or nurse aide trainee, including:

(1) documentation of the Department's investigation, including the nature of the allegation and the evidence that led the Department to confirm the allegation,

(2) the date of the hearing, if requested by the certified nurse aide or nurse aide trainee, and

(3) statement by the individual disputing the finding if the individual chooses to make one.

4. The Department shall include the information specified in subparagraph d of paragraph 3 of this subsection in the registry within ten (10) working days of the substantiating finding and it shall remain in the registry, unless:

a. it has been determined by an administrative law judge, a district court or an appeal court that the finding was in error, or

b. the Board is notified of the death of the certified nurse aide or nurse aide trainee.

5. Upon receipt of an allegation of abuse, exploitation or neglect of a resident or client, or an allegation of misappropriation of resident or client property by a certified nurse aide or nurse aide trainee, the Department shall place a pending notation in the registry until a final determination has been made. If the investigation, or administrative hearing held to determine whether the certified nurse aide or nurse aide trainee is in violation of the law or rules promulgated pursuant thereto, reveals that the abuse, exploitation or neglect, or misappropriation of resident or client property was unsubstantiated, the pending notation shall be removed within twenty-four (24) hours of receipt of notice by the Department.

6. The Department shall, after notice to the individuals involved and a reasonable opportunity for a hearing, make a finding as to the accuracy of the allegations.

7. If the Department after notice and opportunity for hearing determines with clear and convincing evidence that abuse, neglect or exploitation, or misappropriation of resident or client property has occurred and the alleged perpetrator is the person who committed the

prohibited act, notice of the findings shall be sent to the nurse aide and to the district attorney for the county where the abuse, neglect or exploitation, or misappropriation of resident or client property occurred and to the Medicaid Fraud Control Unit of the Attorney General's Office. Notice of ineligibility to work as a nurse aide in a long-term care facility, a residential care facility, assisted living facility, day care facility, or any entity that requires certification of nurse aides, and notice of any further appeal rights shall also be sent to the nurse aide.

8. In any proceeding in which the Department is required to serve notice or an order on an individual, the Department may send written correspondence to the address on file with the registry. If the correspondence is returned and a notation of the United States Postal Service indicates "unclaimed" or "moved" or "refused" or any other nondelivery markings and the records of the registry indicate that no change of address as required by this subsection has been received by the registry, the notice and any subsequent notices or orders shall be deemed by the court as having been legally served for all purposes.

9. The Department shall require that each facility check the nurse aide registry before hiring a person to work as a nurse aide. If the registry indicates that an individual has been found, as a result of a hearing, to be personally responsible for abuse, neglect or exploitation, that individual shall not be hired by the facility.

10. If the state finds that any other individual employed by the facility has neglected, abused, misappropriated property or exploited in a facility, the Department shall notify the appropriate licensing authority and the district attorney for the county where the abuse, neglect or exploitation, or misappropriation of resident or client property occurred.

11. Upon a written request by a certified nurse aide or nurse aide trainee, the Board shall provide within twenty (20) working days all information on the record of the certified nurse aide or nurse aide trainee when a finding of abuse, exploitation or neglect is confirmed and placed in the registry.

12. Upon request and except for the names of residents and clients, the Department shall disclose all of the information relating to the confirmed determination of abuse, exploitation and neglect by the certified nurse aide or nurse aide trainee to the person requesting such information, and may disclose additional information the Department determines necessary.

13. A person who has acted in good faith to comply with state reporting requirements and this section of law shall be immune from liability for reporting allegations of abuse, neglect or exploitation.

F. Each nurse aide trainee shall wear a badge which clearly identifies the person as a nurse aide trainee. Such badge shall be furnished by the facility employing the trainee. The badge shall be

nontransferable and shall include the first and last name of the trainee.

G. 1. For purposes of this section, "feeding assistant" means an individual who is paid to feed residents by a facility or who is used under an arrangement with another agency or organization and meets the requirements cited in 42 CFR Parts 483 and 488.

2. Each facility that employs or contracts employment of a feeding assistant shall maintain a record of all individuals, used by the facility as feeding assistants, who have successfully completed a training course approved by the state for paid feeding assistants.

SECTION 7. This act shall become effective July 1, 2026.

SECTION 8. It being immediately necessary for the preservation of the public peace, health or safety, an emergency is hereby declared to exist, by reason whereof this act shall take effect and be in full force from and after its passage and approval.

COMMITTEE REPORT BY: COMMITTEE ON APPROPRIATIONS AND BUDGET, dated 02/25/2026 - DO PASS, As Amended and Coauthored.